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PHIL 226 - Biomedical Ethics

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August 20, 2023

Narrowing the Health Equity Gap: A Close Look into Healthcare Distribution, Targeted Interventions, and State Obligations in Marginalized Communities

Healthcare access and distribution is a fundamental aspect of society that mirrors its core principles and priorities. The question of whether the state is obliged to provide healthcare or health provider access to its citizens continues to be a complex ethical dilemma. As we acknowledge the undeniable importance of healthcare accessibility, it is important to recognize that the state's responsibilities extend beyond mere provision. The government's role in healthcare is pivotal, as it bears the responsibility to safeguard and enhance the well-being of its citizens. Despite the critical role of state governance in healthcare, the issue of access and distribution remains prevalent, particularly concerning underserved populations. The government's efforts often fall short of addressing the unique healthcare needs of these communities, leading to obvious disparities in health outcomes. Consequently, the urgency to address the health equity gap becomes more evident, further amplifying the need to establish a more inclusive healthcare system. Given that achieving health equity requires targeted policies and interventions that specifically tackle the challenges experienced by minorities and marginalized communities, this paper argues that the government is obliged to prioritize culturally competent care and actively engage community members in the decision-making

processes related to their health. Moreover, this paper counters the libertarian viewpoint which contends that the state bears no responsibility to offer welfare or healthcare services.

In the discourse surrounding health equity in marginalized communities, various viewpoints emerge regarding the role of targeted policies, government obligations, and interventions. A libertarian perspective is provided by Narveson (2011), who emphasizes the difference between negative and positive rights. He contends that people have a negative right to access healthcare, which means they can get medical care without other people getting in the way but that this does not entail a positive right to healthcare provision by the state. Narveson (2011) emphasizes minimal government intervention and the safeguarding of individual liberties. On the other hand, O'Brien (2013) explores the implications of privatized reforms in healthcare, pointing out the reduced state funding and increased reliance on individual resources. She highlights the disparities between private and public healthcare professionals in terms of standards and accountability. Additionally, she examines the global right to health in the context of human rights legislation, stressing governments' responsibility to ensure comprehensive healthcare systems prioritizing quality, accessibility, and equity. Braveman and Gottlieb (2014) further advocate for public health workers and clinicians to address the social determinants of health. They propose strategies beyond individual clinical care that target living and working conditions influencing health, and emphasize the role of healthcare professionals in advocating for health equity at various policy levels. Coursen (2009) then delves into healthcare inequalities' moral and social justice dimensions, linking poverty, inequitable healthcare access, and health outcomes.

The moral obligation to reduce the health disparities that underprivileged people experience forms the basis for this thesis. Due to a number of social, economic, and cultural

variables, minorities and marginalized communities frequently endure severe health disparities. To address these injustices, prioritized interventions are necessary, and community involvement and care that is culturally competent are key strategies. By including communities in decision-making, governments can actively close gaps, fulfilling moral obligations to do so that go beyond merely maximizing welfare (Coursen, 2009).

For successful healthcare interventions, building a foundation of trust and good communication becomes even more important in light of the factors mentioned. Culturally competent care enhances patient-provider trust, leading to open communication and improved treatment adherence. Building trust is crucial in a time when misinformation is pervasive and particularly affects underprivileged people. In order to combat false information and establish long-lasting trusting connections, culturally-sensitive communication techniques are essential. Beyond providing clinical care, physicians and other health professionals have an important role in influencing policies that advance health equity. Their engagement with communities can influence policies that promote health equity, aligning with Braveman and Gottlieb's (2014) assertion that addressing living conditions is central to influencing health outcomes. Moreover, the government must actively work to dismantle historical distrust stemming from mistreatment and neglect in marginalized communities (McGary, 2002).

Continuing the thread of thought, the thesis' emphasis on tailored care seamlessly aligns with preventive healthcare strategies. Culturally relevant measures acknowledge diverse needs and practices, promoting better engagement and treatment adherence. The implementation of culturally competent practices, expanding diversity in the medical profession, and enhancing access in impoverished communities all help close the health literacy gap. These strategies echo the holistic approach to healthcare, accounting for cultural identities and underscoring the

necessity of policies addressing the uniqueness of marginalized communities (Bourassa et al., 2004).

Furthermore, social determinants of health underscore the thesis' emphasis on targeted policies. Given that socioeconomic position influences health, social inequality and its effects on marginalized groups are relevant to the discussion of health inequality and the distribution of healthcare resources (Coursen, 2009). Engaging community members becomes essential in understanding and addressing these determinants, ensuring a comprehensive approach. Recognizing broader societal challenges beyond healthcare bolsters the need for holistic interventions. The influence of social factors on health outcomes is well-established, reinforcing the thesis' argument for state interventions that not only address healthcare access but also underlying social conditions.

While the argument for the government's obligation to prioritize culturally competent care and community engagement in healthcare systems holds merit, opponents may have doubts about how effectively and efficiently such coordination would be carried out. They could question the practical effectiveness of culturally tailored care, suggesting that it may not consistently lead to improved health outcomes. They may argue for medical treatment rooted solely in evidence-based practices, casting doubt on the necessity of catering healthcare to cultural preferences. Moreover, critics might put forth the idea that actively involving community members in decision-making processes could impede swift policy implementation. Streamlined decision-making, from their perspective, is essential for addressing urgent health concerns promptly, particularly in times of crisis. To counter this criticism, it is important to emphasize that health equity stands as a moral obligation that governments and healthcare systems should uphold. Disregarding the distinct challenges faced by marginalized communities

perpetuates systemic inequalities and contradicts principles of social justice and human rights. Governments bear the responsibility to allocate resources justly, ensure transparent governance, and provide equitable healthcare services as part of their international obligations (Gostin, 2010).

Now, if concerns about resource allocation are raised, it is crucial to acknowledge that prioritizing culturally competent care and community engagement can lead to long-term cost savings. Libertarians may argue that healthcare is the responsibility of each person, not the responsibility of society (Crisp, 2017). However, improved health outcomes and reduced health disparities can translate to fewer emergency room visits and hospitalizations, offering potential cost-effective benefits for the overall healthcare system.

Along with recognizing the significance of culturally competent care and community engagement in optimizing cost-effectiveness, it is equally important to consider the interplay between personal autonomy and socioeconomic dynamics in shaping healthcare choices. Personal autonomy is a valid concern from a libertarian viewpoint, but healthcare decisions must also account for the influence of socioeconomic status on informed consent and choice. Tailoring information and support to patients based on their unique circumstances aligns with the principles of patient autonomy and self-determination (Braveman & Gottlieb, 2014). As O'Brien (2013) points out, marginalized groups have a history of having their preferences ignored. Hence, it is important to acknowledge that those requiring these services are often the ones who possess the most accurate understanding of what will truly be effective for their needs.

Ultimately, while the counterarguments highlight that challenges do exist, prioritizing culturally competent care, community engagement, and targeted policies aligns with principles of justice, equity, and the overall improvement of healthcare quality for marginalized communities.

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